

March 31, 2025

This funding opportunity was updated to align with agency priorities. Carefully reread the full funding opportunity and make any needed adjustments to your application prior to submission.

Department of Health and Human Services

Part 1. Overview Information

Participating Organization(s)

National Institutes of Health ([NIH \(http://www.nih.gov/\)](http://www.nih.gov/))

Components of Participating Organizations

National Institute on Drug Abuse ([NIDA \(https://www.drugabuse.gov/\)](https://www.drugabuse.gov/))

Funding Opportunity Title

Development & Testing of Novel Interventions to Improve HIV Prevention, Treatment, and Program Implementation for People Who Use Substances (R34 Clinical Trial Required)

Activity Code

[R34 \(https://grants.nih.gov/grants/funding/ac_search_results.htm?text_curr=r34&Search.x=0&Search.y=0&Search_Type=Activity\)](https://grants.nih.gov/grants/funding/ac_search_results.htm?text_curr=r34&Search.x=0&Search.y=0&Search_Type=Activity) Planning Grant

Announcement Type

Reissue of [PA-21-205 \(https://grants.nih.gov/grants/guide/pa-files/PA-21-205.html\)](https://grants.nih.gov/grants/guide/pa-files/PA-21-205.html)

Related Notices

See [Notices of Special Interest \(https://grants.nih.gov/grants/guide/NOSIs_targetingList.cfm?GuideDocID=42268\)](https://grants.nih.gov/grants/guide/NOSIs_targetingList.cfm?GuideDocID=42268) associated with this funding opportunity

- **January 28, 2026** - NIH Removing AIDS Application Due Dates from NOFOs. See Notice [NOT-OD-26-029 \(https://grants.nih.gov/grants/guide/notice-files/NOT-OD-26-029.html\)](https://grants.nih.gov/grants/guide/notice-files/NOT-OD-26-029.html).
- **March 31, 2025** - This funding opportunity was updated to align with agency priorities. Carefully reread the full funding opportunity and make any needed adjustments to your application prior to submission.
- **April 4, 2024** - Overview of Grant Application and Review Changes for Due Dates on or after January 25, 2025. See Notice [NOT-OD-24-084 \(https://grants.nih.gov/grants/guide/notice-files/NOT-OD-24-084.html\)](https://grants.nih.gov/grants/guide/notice-files/NOT-OD-24-084.html).
- **August 31, 2022** - Implementation Changes for Genomic Data Sharing Plans Included with Applications Due on or after January 25, 2023. See Notice [NOT-OD-22-198 \(https://grants.nih.gov/grants/guide/notice-files/not-od-22-198.html\)](https://grants.nih.gov/grants/guide/notice-files/not-od-22-198.html).
- **August 5, 2022** - Implementation Details for the NIH Data Management and Sharing Policy. See Notice [NOT-OD-22-189 \(https://grants.nih.gov/grants/guide/notice-files/NOT-OD-22-189.html\)](https://grants.nih.gov/grants/guide/notice-files/NOT-OD-22-189.html).

Funding Opportunity Number (FON)

PAR-24-329

Companion Funding Opportunity

None

Number of Applications

See [Section III. 3. Additional Information on Eligibility](#).

Assistance Listing Number(s)

93.279

Funding Opportunity Purpose

This notice of funding opportunity (NOFO) encourages formative research, intervention development, and pilot-testing of interventions for people who use drugs. Primary outcomes of interest include the feasibility, acceptability and safety of novel or adapted interventions that target HIV prevention, treatment or services research. Interventions here may include behavioral, social, or structural approaches, as well as combination biomedical and behavioral approaches that prevent the acquisition or transmission of HIV infection, or improve clinical outcomes for persons living with HIV.

Funding Opportunity Goal(s)

To support basic, clinical, translational, and implementation research in the field of substance use. To develop new knowledge and approaches for the prevention, diagnosis, and treatment of drug use, misuse, and addiction, drug overdose, and related health outcomes, including HIV/AIDS.

Key Dates

Posted Date

January 08, 2025

Open Date (Earliest Submission Date)

April 07, 2025

The following table includes NIH [standard due dates \(https://grants.nih.gov/grants/how-to-apply-application-guide/due-dates-and-submission-policies/due-dates.htm\)](https://grants.nih.gov/grants/how-to-apply-application-guide/due-dates-and-submission-policies/due-dates.htm) marked with an asterisk.

Application Due Dates			Review and Award Cycles		
New	Renewal / Resubmission / Revision (as allowed)	AIDS - New/Renewal/Resubmission/Revision, as allowed	Scientific Merit Review	Advisory Council Review	Earliest Start Date
Not Applicable	Not Applicable	May 07, 2025 *	July 2025	October 2025	December 2025
Not Applicable	Not Applicable	September 07, 2025 *	November 2025	January 2026	April 2026
Not Applicable	Not Applicable	January 07, 2026 *	March 2026	May 2026	July 2026
Not Applicable	Not Applicable	May 07, 2026 *	July 2026	October 2026	December 2026
Not Applicable	Not Applicable	September 07, 2026 *	November 2026	January 2027	April 2027
Not Applicable	Not Applicable	January 07, 2027 *	March 2027	May 2027	July 2027
Not Applicable	Not Applicable	May 07, 2027 *	July 2027	October 2027	December 2027

All applications are due by 5:00 PM local time of applicant organization.

Applicants are encouraged to apply early to allow adequate time to make any corrections to errors found in the application during the submission process by the due date.

Expiration Date

New Date May 8, 2026 per issuance of [NOT-OD-26-029 \(https://grants.nih.gov/grants/guide/notice-files/NOT-OD-26-029.html\)](https://grants.nih.gov/grants/guide/notice-files/NOT-OD-26-029.html). (Original Expiration Date: May 08, 2027)

Due Dates for E.O. 12372

Not Applicable

Required Application Instructions

It is critical that applicants follow the instructions in the Research (R) Instructions in the [How to Apply - Application Guide \(https://grants.nih.gov/grants/guide/url_redirect.php?id=82400\)](https://grants.nih.gov/grants/guide/url_redirect.php?id=82400), except where instructed to do otherwise (in this NOFO or in a Notice from [NIH Guide for Grants and Contracts \(https://grants.nih.gov/grants/guide/url_redirect.php?id=11164\)](https://grants.nih.gov/grants/guide/url_redirect.php?id=11164)).

Conformance to all requirements (both in the Application Guide and the NOFO) is required and strictly enforced. Applicants must read and follow all application instructions in the Application Guide as well as any program-specific instructions noted in Section IV. When the program-specific instructions deviate from those in the Application Guide, follow the program-specific instructions.

Applications that do not comply with these instructions may be delayed or not accepted for review.

There are several options available to submit your application through Grants.gov to NIH and Department of Health and Human Services partners. You **must** use one of these submission options to access the application forms for this opportunity.

1. Use the NIH ASSIST system to prepare, submit and track your application online.

Apply Online Using ASSIST
2. Use an institutional system-to-system (S2S) solution to prepare and submit your application to Grants.gov and [eRA Commons \(https://public.era.nih.gov/commons/\)](https://public.era.nih.gov/commons/) to track your application. Check with your institutional officials regarding availability.
3. Use [Grants.gov \(https://grants.gov/search-grants?oppStatuses=closed|archived|posted|forecasted&fon=PAR-24-329\)](https://grants.gov/search-grants?oppStatuses=closed|archived|posted|forecasted&fon=PAR-24-329) Workspace to prepare and submit your application and [eRA Commons \(http://public.era.nih.gov/commons/\)](http://public.era.nih.gov/commons/) to track your application.

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Part 2. Full Text of Announcement

Section I. Notice of Funding Opportunity Description

Background

Declines in new HIV cases have been observed among adults and adolescents in recent years, domestically and internationally. Nonetheless, the magnitude of decline in the US suggests that advancements in prevention and treatment as well as efforts to increase the reach of effective interventions have been insufficient to decisively end the HIV epidemic. Domestic outbreaks of HIV have been identified among people who inject drugs (PWID) and stimulant drug use remains a substantial attributable risk among key populations such as men who have sex with men (MSM). Polysubstance use always has been a feature of drug use; the increased use of stimulants among opioid users in the US poses new challenges to HIV prevention and care in the US. Internationally, HIV epidemics related to stimulant and/or opioid use have been expanding in Southeast Asia and sub-Saharan Africa while epidemics related to opioid injection drug use have continued in Eastern Europe and Central Asia, and substance use-related HIV epidemics are evident in other regions of the world. Tools for HIV prevention and care have appeared in a variety of international settings that offer opportunities to inform HIV-substance use epidemics in the US and elsewhere.

Progress has been made in the development and implementation of efficacious HIV prevention and treatment interventions, including seek/test/treat/retain approaches, more sensitive HIV testing technologies, and new pre-exposure prophylaxis (PrEP) formulations. The Ending the Epidemic initiative and other federally-sponsored activities have attempted to broaden the reach, intensity, and integration of these interventions both domestically, as well as abroad, through the U.S. President's Emergency Plan for AIDS Relief (PEPFAR) and additional initiatives exist in other countries. Nonetheless, persons in key populations still need better access to HIV testing, preventive interventions, optimal HIV care, and effective management of co-morbidities such as substance use disorders. A great many people living with HIV (PLWH) are not adequately linked, engaged or retained in care so that treatment adherence may be insufficient to achieve and maintain viral suppression. Attention to substance use, both episodic and ongoing, and its impact on HIV risk, prevention, treatment, and retention need further attention.

Novel approaches are needed to ensure that interventions address the particular needs of people who use substances (PWUS). These interventions need to address the range of settings where PWUS can be found and the service delivery systems available to them. PLWH often have needs that differ from those who are not living with HIV, and interventions for PLWH are needed that promote continued HIV medication adherence, consistent linkage to HIV medical care, and durable viral suppression. Regardless of HIV status, the promotion of a quality of life that is free of problematic substance use, co-occurring disorders (infectious disease and psychiatric), stigma, and discrimination should be considered as intervention objectives.

This notice of funding opportunity (NOFO) provides resources to support (a) pilot or feasibility studies of new or adapted interventions to prevent HIV infection among populations where substance use is a primary or contributing factor; (b) pilot or feasibility studies of new interventions to improve the care of HIV infection among populations where substance use is prevalent, including interventions that integrate treatment for substance use disorders and HIV infection; or (c) pilot or feasibility studies to reduce barriers, including structural barriers, to HIV prevention and care and increase the scale, uptake, delivery, sustainability and/or quality of HIV prevention or care interventions that already have evidence of efficacy. The Ending the HIV Epidemic initiative has promoted status neutral approaches, but HIV incidence and prevalence in various populations may require particular attention to PLWH or without HIV, while also insuring that needs of all populations are addressed. If successful, these investigations should lead to applications using the R01 or similar mechanisms for evaluating large scale efficacy or effectiveness. This NOFO does not support projects that focus only on development of intervention protocols, manuals, or the standardization of protocols as interventions.

Substance use should be integrated into study aims and hypotheses in ways that reflect its roles in HIV prevention and care continua. The full range of substance use should be considered including substance use disorders and problematic episodic use as well as a complete range of relevant substances and all modes of substance administration (e.g., injection, non-injection). Domestic and overseas intervention studies will be considered with particular attention to key populations who have disproportionate burden of HIV infection, which may vary by location. Adaptation of interventions with known efficacy or effectiveness to novel populations, settings, or systems of delivery should contribute to the understanding of how to appropriately adapt and tailor interventions to new circumstances.

Applications to this NOFO are not required to present pilot data in support of the proposed hypotheses and aims, but should provide a strong scientific premise that includes a well-supported theory of change or logic model and associated hypotheses. Applications will not be penalized for a lack of preliminary data supporting the proposed hypotheses and aims. Applicants are encouraged to provide strong evidence of their capability to conduct the proposed study by documenting the availability of needed resources, the training and experience of the investigator team, and/or the conduct of related studies.

Even at the pilot stage of development, interventions should consider sustainability and scalability. Applicants must engage relevant end users in study conceptualization, design, execution, and interpretation to maximize the acceptability, feasibility, scalability and sustainability of prevention intervention or services being studied. End user is broadly defined and includes both those who may pay for and/or implement an intervention and those who are meant to benefit from the intervention. Examples of end users include: policymakers, decision makers at national and more local levels, practitioners, intervention implementers, members of key populations, and community members. End users may be incorporated into the proposed research as members of the study team (e.g., PIs or co-Is), consultants, and/or advisory board members.

Applicants should consult the NIH HIV priorities <https://www.oar.nih.gov/hiv-policy-and-research/research-priorities> (<https://www.oar.nih.gov/hiv-policy-and-research/research-priorities>) as well as the National Institute on Drug Abuse's (NIDA's) HIV priorities <https://nida.nih.gov/about-nida/organization/offices/hiv-research-program-hrp/about-hiv-research-program> (<https://nida.nih.gov/about-nida/organization/offices/hiv-research-program-hrp/about-hiv-research-program>) as they prepare their applications.

Specific Areas of Research Interest

Examples of research questions that may help move the HIV intervention field forward, under this NOFO, include (but are not limited to) the following:

- Studies to Advance HIV Prevention:
 - Develop and test approaches to enhance the long-term maintenance of HIV risk reduction, including new PrEP formulations and delivery mechanisms (e.g., injectable PrEP).
 - Studies that test ways to combine prevention interventions of known efficacy, including adaptive designs that test different sequences of intervention implementation to improve outcomes.

- Interventions that incorporate a syndemic approach to HIV prevention including substance use and co-morbidities such as psychiatric disorder or infectious disease, as well as conditions such as intimate partner violence, childhood sexual abuse, stigma, discrimination, or substance use.
- Studies using novel technology or technology-enabled approaches to recruit, enroll, and retain members of key populations who are difficult to reach by other means.
- Studies to identify and test interventions that integrate HIV prevention into settings where these interventions often are not offered (e.g., primary care, criminal justice, drug treatment, youth-serving organizations).
- Implementation research that focuses on new PrEP formulations or combination biological/behavioral prevention modalities among substance using populations.
- Development and testing of HIV/AIDS prevention interventions that incorporate reduction in stigma toward persons at elevated risk for HIV/AIDS acquisition.
- Develop and test interventions that meet prevention needs of persons whose HIV risk persists despite current or previous engagement in evidence-based interventions (e.g., testing, behavioral risk reduction, PrEP). The use of innovative approaches such as adaptive designs is encouraged.
- Studies that take advantage of opportunities to evaluate the implementation and short-term effects of new policies, settings, or health care financing systems that are designed to prevent HIV and related consequences of substance use.
- Pilot or feasibility studies that take initial steps toward implementation of structural or community-level prevention interventions.
- Studies to Advance Treatment and Adherence Research:
 - Studies to optimize the provision of brief, evidence-based interventions to treat or reduce substance use among HIV at-risk populations, while increasing uptake and utilization of HIV prevention services (e.g., testing, PrEP).
 - Studies which increase the likelihood of achieving and/or maintaining viral suppression of HIV infection.
 - Studies to develop novel approaches for augmenting the impact of interventions of known efficacy to promote HIV treatment adherence and persistence among PWUS.
 - Feasibility and acceptability studies that incorporate technological tool(s) to promote patient engagement in HIV care (including early and efficient linkage to care; treatment adherence behaviors; and long-term retention in care) among drug using populations living with HIV.
 - Early stage trials of integrated substance use disorder treatment (behavioral or pharmacological) and HIV prevention interventions that address HIV risk practices (sexual and substance use behavior) and promote uptake of prevention services (e.g., testing, PrEP) in key populations.
 - Development and testing of novel telehealth strategies for research in individuals with HIV and substance use disorders, including tailored delivery to affected populations and settings (e.g., non-clinical settings, rural communities with constrained broadband).
 - Development of non-pharmacological therapies (cognitive behavioral approaches, integrative approaches such as mindfulness) for use in conjunction with medication-based drug treatment to improve HIV clinical outcomes in the context of substance use disorder.
- Studies to Advance Program Implementation and Health Services Research:
 - Improving access and quality of HIV prevention or treatment services for PWUS and also are living with HIV from vulnerable populations including women, adolescents, racial/ethnic minorities, and those with co-occurring disorders (e.g., psychiatric disorder, viral hepatitis).
 - Pilot testing of interventions to improve the delivery of substance use treatment services for HIV at-risk populations in non-traditional settings such as schools, criminal justice settings, primary care practices, and AIDS service organizations.
 - Develop and pilot test interventions that provide integrated, holistic approaches to service delivery to PLWH who also have comorbid substance use and/or mental health disorders (e.g., depression, PTSD, schizophrenia, and other disorders).
 - Developing and evaluating innovative implementation strategies and novel service delivery strategies for the treatment and care for vulnerable PLWH.
 - Interventions that target and address structural, organizational or policy interventions to barriers and facilitators of science-based HIV/AIDS treatment and/or prevention.
 - Implementation research that helps overcome barriers to achieving adherence or viral suppression with navigation or case management approaches among PWUS.
 - Developing interventions that influence organizational structure, climate, and culture in order to promote organizational readiness and capacity for dissemination and adoption of evidence-based practices.
 - Evaluations of the adaptation process, and subsequent effectiveness of evidence-based interventions when implemented in real-world community and clinical settings.
 - Pilot or feasibility studies that take initial steps toward implementation of structural approaches (e.g. structural determinants of health) to improving the quality and appropriateness of services and/or access and utilization of services.
 - Investigators are encouraged, when appropriate, to utilize the NIDA Clinical Trials Network (CTN) (<http://www.drugabuse.gov/CTN/Index.htm>) (<http://www.drugabuse.gov/CTN/Index.htm>) infrastructure as a platform for their studies.

Applications Not Responsive to this NOFO

The R34 mechanism is for clinical trial planning projects. The following types of studies are not responsive to this NOFO and will not be reviewed:

- Applications proposing only to develop manuals, protocols or the standardization of protocols.
- Applications proposing a focus on alcohol use without the inclusion of other substances.
- Applications that do not describe methods for engaging relevant end users of data in the development of the project.

Special Considerations

NIDA applicants are strongly encouraged to review the guidelines and adhere to the requirements applicable to their research listed in the [Special Considerations for NIDA Funding Opportunities and Awards](https://gcc02.safelinks.protection.outlook.com/?url=https%3A%2F%2Fwww.drugabuse.gov%2Ffunding%2Fspecial-considerations-for-nida-funding&data=05%7C01%7C%Yvonne.walker%40nih.gov%7C2b30605ae50740cd31fb08dad6eab2e5%7C14b77578977342d58507251ca2dc2b06%7C0%7C0%7C6380586007528955) (<https://gcc02.safelinks.protection.outlook.com/?url=https%3A%2F%2Fwww.drugabuse.gov%2Ffunding%2Fspecial-considerations-for-nida-funding&data=05%7C01%7C%Yvonne.walker%40nih.gov%7C2b30605ae50740cd31fb08dad6eab2e5%7C14b77578977342d58507251ca2dc2b06%7C0%7C0%7C6380586007528955>). Upon award, these considerations will be included in the Notice of Grant Award.

Investigators proposing NIH-defined clinical trials may refer to the [Research Methods Resources](https://researchmethodsresources.nih.gov/) (<https://researchmethodsresources.nih.gov/>) website for information about developing statistical methods and study designs.

See [Section VIII. Other Information](#) for award authorities and regulations.

Section II. Award Information

Funding Instrument

Grant: A financial assistance mechanism providing money, property, or both to an eligible entity to carry out an approved project or activity.

Application Types Allowed

New
Resubmission

The [OER Glossary \(//grants.nih.gov/grants/guide/uri_redirect.php?id=11116\)](https://grants.nih.gov/grants/guide/uri_redirect.php?id=11116) and the How to Apply Application Guide provide details on these application types. Only those application types listed here are allowed for this NOFO.

Clinical Trial?

Required: Only accepting applications that propose clinical trial(s).

[Need help determining whether you are doing a clinical trial? \(https://grants.nih.gov/grants/guide/uri_redirect.php?id=82370\)](https://grants.nih.gov/grants/guide/uri_redirect.php?id=82370)

Funds Available and Anticipated Number of Awards

The number of awards is contingent upon NIH appropriations and the submission of a sufficient number of meritorious applications.

Award Budget

Applicants may request direct costs of up to \$450,000 for three years. Although variations from year to year are permissible, in no case may any year be more than \$225,000 in direct costs, and total direct costs for the entire project period may not exceed \$450,000.

Award Project Period

The maximum period is 3 years.

NIH grants policies as described in the [NIH Grants Policy Statement \(//grants.nih.gov/grants/guide/uri_redirect.php?id=11120\)](https://grants.nih.gov/grants/guide/uri_redirect.php?id=11120) will apply to the applications submitted and awards made from this NOFO.

Section III. Eligibility Information

1. Eligible Applicants

Eligible Organizations

Higher Education Institutions

- Public/State Controlled Institutions of Higher Education
- Private Institutions of Higher Education

Nonprofits Other Than Institutions of Higher Education

- Nonprofits with 501(c)(3) IRS Status (Other than Institutions of Higher Education)
- Nonprofits without 501(c)(3) IRS Status (Other than Institutions of Higher Education)

For-Profit Organizations

- Small Businesses
- For-Profit Organizations (Other than Small Businesses)

Local Governments

- State Governments
- County Governments
- City or Township Governments
- Special District Governments
- Indian/Native American Tribal Governments (Federally Recognized)
- Indian/Native American Tribal Governments (Other than Federally Recognized).

Federal Government

- Eligible Agencies of the Federal Government
- U.S. Territory or Possession

Other

- Independent School Districts
- Public Housing Authorities/Indian Housing Authorities
- Native American Tribal Organizations (other than Federally recognized tribal governments)
- Faith-based or Community-based Organizations
- Regional Organizations
- Non-domestic (non-U.S.) Entities (Foreign Organizations).

Foreign Organizations

Non-domestic (non-U.S.) Entities (Foreign Organizations) **are** eligible to apply.

Non-domestic (non-U.S.) components of U.S. Organizations **are** eligible to apply.

Foreign components, as [defined in the NIH Grants Policy Statement \(//grants.nih.gov/grants/guide/uri_redirect.php?id=11118\)](https://grants.nih.gov/grants/guide/uri_redirect.php?id=11118), **are** allowed.

Required Registrations

Applicant Organizations

Applicant organizations must complete and maintain the following registrations as described in the How to Apply-Application Guide to be eligible to apply for or receive an award. All registrations must be completed prior to the application being submitted. Registration can take 6 weeks or more, so applicants should begin the registration process as soon as

possible. Failure to complete registrations in advance of a due date is not a valid reason for a late submission, please reference the [NIH Grants Policy Statement Section 2.3.9.2 Electronically Submitted Applications](#) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=82423).

- **System for Award Management (SAM)** – (https://grants.nih.gov/grants/guide/uri_redirect.php?id=82390) Applicants must complete and maintain an active registration, **which requires renewal at least annually**. The renewal process may require as much time as the initial registration. SAM registration includes the assignment of a Commercial and Government Entity (CAGE) Code for domestic organizations which have not already been assigned a CAGE Code.
 - **NATO Commercial and Government Entity (NCAGE) Code** (https://grants.nih.gov/grants/guide/uri_redirect.php?id=11176) – Foreign organizations must obtain an NCAGE code (in lieu of a CAGE code) in order to register in SAM.
 - **Unique Entity Identifier (UEI)** - A UEI is issued as part of the SAM.gov registration process. The same UEI must be used for all registrations, as well as on the grant application.
- **eRA Commons** (https://grants.nih.gov/grants/guide/uri_redirect.php?id=11123) - Once the unique organization identifier is established, organizations can register with eRA Commons in tandem with completing their Grants.gov registration; all registrations must be in place by time of submission. eRA Commons requires organizations to identify at least one Signing Official (SO) and at least one Program Director/Principal Investigator (PD/PI) account in order to submit an application.
- **Grants.gov** (https://grants.nih.gov/grants/guide/uri_redirect.php?id=82300) – Applicants must have an active SAM registration in order to complete the Grants.gov registration.

Program Directors/Principal Investigators (PD(s)/PI(s))

All PD(s)/PI(s) must have an eRA Commons account. PD(s)/PI(s) should work with their organizational officials to either create a new account or to affiliate their existing account with the applicant organization in eRA Commons. If the PD/PI is also the organizational Signing Official, they must have two distinct eRA Commons accounts, one for each role. Obtaining an eRA Commons account can take up to 2 weeks.

Eligible Individuals (Program Director/Principal Investigator)

Any individual(s) with the skills, knowledge, and resources necessary to carry out the proposed research as the Program Director(s)/Principal Investigator(s) (PD(s)/PI(s)) is invited to work with their organization to develop an application for support.

For institutions/organizations proposing multiple PDs/PIs, visit the Multiple Program Director/Principal Investigator Policy and submission details in the Senior/Key Person Profile (Expanded) Component of the How to Apply- Application Guide.

2. Cost Sharing

This NOFO does not require cost sharing as defined in the [NIH Grants Policy Statement Section 1.2 Definition of Terms](#) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=11126).

3. Additional Information on Eligibility

Number of Applications

Applicant organizations may submit more than one application, provided that each application is scientifically distinct.

The NIH will not accept duplicate or highly overlapping applications under review at the same time, per [NIH Grants Policy Statement Section 2.3.7.4 Submission of Resubmission Application](#) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=82415). This means that the NIH will not accept:

- A new (A0) application that is submitted before issuance of the summary statement from the review of an overlapping new (A0) or resubmission (A1) application.
- A resubmission (A1) application that is submitted before issuance of the summary statement from the review of the previous new (A0) application.
- An application that has substantial overlap with another application pending appeal of initial peer review (see [NIH Grants Policy Statement 2.3.9.4 Similar, Essentially Identical, or Identical Applications](#) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=82423)).

Section IV. Application and Submission Information

1. Requesting an Application Package

The application forms package specific to this opportunity must be accessed through ASSIST, Grants.gov Workspace or an institutional system-to-system solution. Links to apply using ASSIST or Grants.gov Workspace are available in Part 1 of this NOFO. See your administrative office for instructions if you plan to use an institutional system-to-system solution.

2. Content and Form of Application Submission

It is critical that applicants follow the instructions in the Research (R) Instructions in the [How to Apply - Application Guide](#) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=82400), except where instructed in this notice of funding opportunity to do otherwise. Conformance to the requirements in the Application Guide is required and strictly enforced. Applications that are out of compliance with these instructions may be delayed or not accepted for review.

Page Limitations

All page limitations described in the [How to Apply- Application Guide](#) (<https://grants.nih.gov/grants/how-to-apply-application-guide.html>) and the [Table of Page Limits](#) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=61134) must be followed.

Instructions for Application Submission

The following section supplements the instructions found in the [How to Apply- Application Guide](#) (<https://grants.nih.gov/grants/how-to-apply-application-guide.html>) and should be used for preparing an application to this NOFO.

SF424(R&R) Cover

All instructions in the How to Apply- Application Guide must be followed.

SF424(R&R) Project/Performance Site Locations

All instructions in the How to Apply-Application Guide must be followed.

SF424(R&R) Other Project Information

All instructions in the How to Apply-Application Guide must be followed.

SF424(R&R) Senior/Key Person Profile

All instructions in the How to Apply- Application Guide must be followed.

R&R or Modular Budget

All instructions in the How to Apply- Application Guide must be followed.

R&R Subaward Budget

All instructions in the How to Apply-Application Guide must be followed.

PHS 398 Cover Page Supplement

All instructions in the How to Apply- Application Guide must be followed.

PHS 398 Research Plan

All instructions in the How to Apply- Application Guide must be followed, with the following additional instructions:

Research Strategy

The Research Strategy should describe the significance of the proposed research in the context of the future efficacy, effectiveness, or implementation trial it is meant to inform.

Partnerships and collaborations

Applicants are required to meaningfully engage end users and/or community experts in their research. These partnership(s) should be documented in the Research Strategy section to describe the end-user/expert's involvement in development, design and testing of the intervention, including establishing a Community Advisory Board or other advisory boards as appropriate. If using Community Based Participatory Research (CBPR) research methods, these must be appropriately documented. The scientific, logistic, and organizational responsibilities of each of the collaborators for each aspect of the proposed project should be described. If building upon existing partnerships, the application should also describe the history of the partners in collaborating on prior research projects or related activities.

Approach

Applications to this NOFO are not required to present preliminary/pilot data in support of the proposed approaches, hypotheses and aims; rather, a well-defined theory of change, logic model, or cultural source and associated hypotheses must be included.

Specific justification must be provided for the necessity of all planning, intervention development, and services research activities proposed, including cultural adaptation of interventions, measures, or implementation strategies that have been used or validated with other populations or in other settings. Appropriate justifications of scientific gap areas must be provided in the applications.

Whenever possible, evidence of the feasibility of the planning and research activities proposed should be included. This evidence of feasibility could include prior collaborations between the investigators and between the research team and study partners, prior experience working with the study population, and prior experience with the measures, methods, and analysis approaches to be used or tested in the research.

When the research involves intervention development, adaptation, or acceptability and feasibility testing, the intervention (or the process of its initial development) must be described, including the theoretical and empirical evidence informing the intervention, the proposed mechanisms of action of the intervention, the health outcomes the intervention is meant to impact, and any relevant protective factors or risk factors that the intervention acts on or through for its hypothesized effects.

The pilot studies proposed under this NOFO may or may not require a comparison condition. If included, the application must describe and justify the nature of any comparison/control group (e.g., whether the comparison group will receive information only, minimal treatment, or delayed intervention or some other design). Applicants should clearly describe how the specific pilot study design chosen will support the goals of the research and generate the data needed to inform the future efficacy, effectiveness or implementation trial. Special attention should be paid to the justification of sample sizes and applicants should not propose analyses for which the study is not adequately powered (e.g., if the study is not powered for efficacy testing, applicants should not propose to report on "preliminary efficacy").

The application must describe how the approach taken will support the future sustainability of the intervention, and any implications for implementing or disseminating the intervention if it is found to be effective in the future trial. When appropriate, the study should include plans for assessing fidelity of intervention and/or protocol implementation. Plans for manualization or other standardization practices should also be included.

Resource Sharing Plan: Individuals are required to comply with the instructions for the Resource Sharing Plans as provided in the How to Apply- Application Guide.

Other Plan(s):

All instructions in the How to Apply-Application Guide must be followed, with the following additional instructions:

- All applicants planning research (funded or conducted in whole or in part by NIH) that results in the generation of scientific data are required to comply with the instructions for the Data Management and Sharing Plan. All applications, regardless of the amount of direct costs requested for any one year, must address a Data Management and Sharing Plan.

Appendix: Only limited Appendix materials are allowed. Follow all instructions for the Appendix as described in the How to Apply- Application Guide.

- No publications or other material, with the exception of blank questionnaires or blank surveys, may be included in the Appendix.

PHS Human Subjects and Clinical Trials Information

When involving human subjects research, clinical research, and/or NIH-defined clinical trials (and when applicable, clinical trials research experience) follow all instructions for the PHS Human Subjects and Clinical Trials Information form in the How to Apply- Application Guide, with the following additional instructions:

If you answered "Yes" to the question "Are Human Subjects Involved?" on the R&R Other Project Information form, you must include at least one human subjects study record using the **Study Record: PHS Human Subjects and Clinical Trials Information** form or **Delayed Onset Study** record.

Study Record: PHS Human Subjects and Clinical Trials Information

All instructions in the How to Apply- Application Guide must be followed.

Delayed Onset Study

Note: [Delayed onset \(https://grants.nih.gov/grants/glossary.htm#DelayedOnsetStudy\)](https://grants.nih.gov/grants/glossary.htm#DelayedOnsetStudy) does NOT apply to a study that can be described but will not start immediately (i.e., delayed start). All instructions in the How to Apply-Application Guide must be followed.

PHS Assignment Request Form

All instructions in the How to Apply-Application Guide must be followed.

Foreign Organizations

Foreign (non-U.S.) institutions must follow policies described in the [NIH Grants Policy Statement \(//grants.nih.gov/grants/guide/uri_redirect.php?id=11137\)](https://grants.nih.gov/grants/guide/uri_redirect.php?id=11137), and procedures for foreign institutions described throughout the How to Apply-Application Guide.

3. Unique Entity Identifier and System for Award Management (SAM)

See Part 2, Section III.1 for information regarding the requirement for obtaining a unique entity identifier and for completing and maintaining active registrations in System for Award Management (SAM), NATO Commercial and Government Entity (NCAGE) Code (if applicable), eRA Commons, and Grants.gov.

4. Submission Dates and Times

[Part I. Overview Information](#) contains information about Key Dates and times. Applicants are encouraged to submit applications before the due date to ensure they have time to make any application corrections that might be necessary for successful submission. When a submission date falls on a weekend or [Federal holiday](#) (https://grants.nih.gov/grants/guide/url_redirect.php?id=82380), the application deadline is automatically extended to the next business day.

Organizations must submit applications to [Grants.gov](#) (https://grants.nih.gov/grants/guide/url_redirect.php?id=11128) (the online portal to find and apply for grants across all Federal agencies). Applicants must then complete the submission process by tracking the status of the application in the [eRA Commons](#) (https://grants.nih.gov/grants/guide/url_redirect.php?id=11123), NIH's electronic system for grants administration. NIH and Grants.gov systems check the application against many of the application instructions upon submission. Errors must be corrected and a changed/corrected application must be submitted to Grants.gov on or before the application due date and time. If a Changed/Corrected application is submitted after the deadline, the application will be considered late. Applications that miss the due date and time are subjected to the [NIH Grants Policy Statement Section 2.3.9.2 Electronically Submitted Applications](#) (https://grants.nih.gov/grants/guide/url_redirect.php?id=82423).

Applicants are responsible for viewing their application before the due date in the eRA Commons to ensure accurate and successful submission.

Information on the submission process and a definition of on-time submission are provided in the How to Apply- Application Guide.

5. Intergovernmental Review (E.O. 12372)

This initiative is not subject to [intergovernmental review](#). (https://grants.nih.gov/grants/policy/nihgps/html5/section_10/10.10.1_executive_orders.htm)

6. Funding Restrictions

All NIH awards are subject to the terms and conditions, cost principles, and other considerations described in the [NIH Grants Policy Statement](#) (https://grants.nih.gov/grants/guide/url_redirect.php?id=11120).

Pre-award costs are allowable only as described in the [NIH Grants Policy Statement Section 7.9.1 Selected Items of Cost](#). (https://grants.nih.gov/grants/guide/url_redirect.php?id=11143)

7. Other Submission Requirements and Information

Applications must be submitted electronically following the instructions described in the [How to Apply - Application Guide](#) (https://grants.nih.gov/grants/guide/url_redirect.php?id=82400). Paper applications will not be accepted.

Applicants must complete all required registrations before the application due date. [Section III. Eligibility Information](#) contains information about registration.

For assistance with your electronic application or for more information on the electronic submission process, visit [How to Apply – Application Guide](#) (https://grants.nih.gov/grants/guide/url_redirect.php?id=82400). If you encounter a system issue beyond your control that threatens your ability to complete the submission process on-time, you must follow the [Dealing with System Issues](#) (<https://grants.nih.gov/grants/how-to-apply-application-guide/due-dates-and-submission-policies/dealing-with-system-issues.htm>) guidance. For assistance with application submission, contact the Application Submission Contacts in [Section VII](#).

Important reminders:

All PD(s)/PI(s) must include their eRA Commons ID in the Credential field of the Senior/Key Person Profile form. Failure to register in the Commons and to include a valid PD/PI Commons ID in the credential field will prevent the successful submission of an electronic application to NIH. See Section III of this NOFO for information on registration requirements.

The applicant organization must ensure that the unique entity identifier provided on the application is the same identifier used in the organization's profile in the eRA Commons and for the System for Award Management. Additional information may be found in the [How to Apply - Application Guide](#) (https://grants.nih.gov/grants/guide/url_redirect.php?id=82400).

See [more tips](#) (https://grants.nih.gov/grants/guide/url_redirect.php?id=11146) for avoiding common errors.

Upon receipt, applications will be evaluated for completeness and compliance with application instructions by the Center for Scientific Review and responsiveness by NIDA, NIH. Applications that are incomplete, non-compliant and/or nonresponsive will not be reviewed.

Mandatory Disclosure

Recipients or subrecipients must submit any information related to violations of federal criminal law involving fraud, bribery, or gratuity violations potentially affecting the federal award. See Mandatory Disclosures, [2 CFR 200.113](#) (<https://www.ecfr.gov/current/title-2/subtitle-A/chapter-II/part-200/subpart-B/section-200.113>) and [NIH Grants Policy Statement Section 4.1.35](#) (https://grants.nih.gov/grants/policy/nihgps/html5/section_4/4.1.35_mandatory_disclosures.htm).

Send written disclosures to the NIH Chief Grants Management Officer listed on the Notice of Award for the IC that funded the award and to the [HHS Office of Inspector Grant Self Disclosure Program](#) (<https://oig.hhs.gov/compliance/self-disclosure-info/hhs-oig-grant-self-disclosure-program/>) at grantdisclosures@oig.hhs.gov (<mailto:grantdisclosures@oig.hhs.gov>).

Post Submission Materials

Applicants are required to follow the instructions for post-submission materials, as described in [the policy](#) (https://grants.nih.gov/grants/guide/url_redirect.php?id=82299)

Section V. Application Review Information

1. Criteria

Only the review criteria described below will be considered in the review process. Applications submitted to the NIH in support of the [NIH mission](#) (https://grants.nih.gov/grants/guide/url_redirect.php?id=11149) are evaluated for scientific and technical merit through the NIH peer review system.

Overall Impact

Reviewers will provide an overall impact score to reflect their assessment of the likelihood for the project to exert a sustained, powerful influence on the research field(s) involved, in consideration of the following scored review criteria and additional review criteria (as applicable for the project proposed). An application does not need to be strong in all categories to be judged likely to have a major scientific impact.

Review Criteria

Reviewers will consider Factors 1, 2 and 3 in the determination of scientific merit, and in providing an overall impact score. In addition, Factors 1 and 2 will each receive a separate factor score.

Factor 1: Importance of the Research

Significance

- Evaluate the importance of the proposed research in the context of current scientific challenges and opportunities, either for advancing knowledge within the field, or more broadly. Assess whether the application addresses an important gap in knowledge in the field, would solve a critical problem, or create a valuable conceptual or technical advance.
- Evaluate the rationale for undertaking the study, the rigor of the scientific background for the work (e.g. prior literature and/or preliminary data) and whether the scientific background justifies the proposed study.

Innovation

- Evaluate the extent to which innovation influences the importance of undertaking the proposed research. Note that while technical or conceptual innovation can influence the importance of the proposed research, a project that is not applying novel concepts or approaches may be of critical importance for the field.
- Evaluate whether the proposed work applies novel concepts, methods or technologies, or uses existing concepts, methods, technologies in novel ways, to enhance the overall impact of the project.

Factor 2. Rigor and Feasibility**Approach**

- Evaluate the scientific quality of the proposed work. Evaluate the likelihood that compelling, reproducible findings will result (rigor) and assess whether the proposed studies can be done well and within the timeframes proposed (feasibility).

Rigor:

- Evaluate the potential to produce unbiased, reproducible, robust data.
- Evaluate the rigor of the experimental design and whether appropriate controls are in place.
- Evaluate whether the sample size is sufficient and well-justified.
- Assess the quality of the plans for analysis, interpretation, and reporting of results.
- Evaluate whether the investigators presented adequate plans to address relevant biological variables, such as sex or age, in the design, analysis, and reporting.
- For applications involving human subjects or vertebrate animals, also evaluate:
 - the rigor of the intervention or study manipulation (if applicable to the study design).
 - whether outcome variables are justified.
 - whether the results will be generalizable or, in the case of a rare disease/special group, relevant to the particular subgroup.
 - whether the sample is appropriate and sufficiently diverse to address the proposed question(s).
- For applications involving human subjects, including clinical trials, assess the adequacy of inclusion plans as appropriate for the scientific goals of the research. Considerations of appropriateness may include disease/condition/behavior incidence, prevalence, or population burden, population representation, and/or current state of the science.

Feasibility:

- Evaluate whether the proposed approach is sound and achievable, including plans to address problems or new challenges that emerge in the work. For proposed studies in which feasibility may be less certain, evaluate whether the uncertainty is balanced by the potential for major advances.
- For applications involving human subjects, including clinical trials, evaluate the adequacy and feasibility of the plan to recruit and retain a study population that appropriately models the target population. Additionally, evaluate the likelihood of successfully achieving the proposed enrollment based on age, race, ethnicity, and sex.
- For clinical trial applications, evaluate whether the study timeline and milestones are feasible.

Specific to this NOFO:

- Evaluate the extent that the application involves meaningful engagement of end users and/or community experts in the research.
- Evaluate the appropriateness of the proposed conceptual model.
- Evaluate the justification for the theoretical and empirical evidence informing the intervention.
- Evaluate the appropriateness of the methodology for any proposed NIH-defined clinical trial, including the appropriateness of any proposed comparison condition.

Factor 3. Expertise and Resources**Investigator(s)**

- Evaluate whether the investigator(s) have demonstrated background, training, and expertise, as appropriate for their career stage, to conduct the proposed work. For Multiple Principal Investigator (MPI) applications, assess the quality of the leadership plan to facilitate coordination and collaboration.

Environment

- Evaluate whether the institutional resources are appropriate to ensure the successful execution of the proposed work.

Additional Review Criteria

As applicable for the project proposed, reviewers will consider the following additional items while determining scientific and technical merit, but will not give criterion scores for these items, and should consider them in providing an overall score.

Protections for Human Subjects

For research that involves human subjects but does not involve one of the categories of research that are exempt under 45 CFR Part 46, evaluate the justification for involvement of human subjects and the proposed protections from research risk relating to their participation according to the following five review criteria: 1) risk to subjects, 2) adequacy of protection against risks, 3) potential benefits to the subjects and others, 4) importance of the knowledge to be gained, and 5) data and safety monitoring for clinical trials.

For research that involves human subjects and meets the criteria for one or more of the categories of research that are exempt under 45 CFR Part 46, evaluate: 1) the justification for the exemption, 2) human subjects involvement and characteristics, and 3) sources of materials. For additional information on review of the Human Subjects section, please refer to the [Guidelines for the Review of Human Subjects \(https://grants.nih.gov/grants/guide/ir/ir_guidelines.html\)](https://grants.nih.gov/grants/guide/ir/ir_guidelines.html).

Vertebrate Animals

When the proposed research includes Vertebrate Animals, evaluate the involvement of live vertebrate animals according to the following criteria: (1) description of proposed procedures involving animals, including species, strains, ages, sex, and total number to be used; (2) justifications for the use of animals versus alternative models and for the appropriateness of the species proposed; (3) interventions to minimize discomfort, distress, pain and injury; and (4) justification for euthanasia method if NOT consistent with the AVMA Guidelines for the Euthanasia of Animals. For additional information on review of the Vertebrate Animals section, please refer to the [Worksheet for Review of the Vertebrate Animals Section \(https://grants.nih.gov/grants/guide/ir/ir_guidelines.html\)](https://grants.nih.gov/grants/guide/ir/ir_guidelines.html).

Biohazards

When the proposed research includes Biohazards, evaluate whether specific materials or procedures that will be used are significantly hazardous to research personnel and/or the environment, and whether adequate protection is proposed.

Resubmissions

As applicable, evaluate the application as now presented.

Renewals

Not applicable.

Revisions

Not applicable.

Additional Review Considerations

As applicable for the project proposed, reviewers will consider each of the following items, but will not give scores for these items, and should not consider them in providing an overall impact score.

Authentication of Key Biological and/or Chemical Resources:

For projects involving key biological and/or chemical resources, evaluate the brief plans proposed for identifying and ensuring the validity of those resources.

Budget and Period of Support

Evaluate whether the budget and the requested period of support are fully justified and reasonable in relation to the proposed research.

2. Review and Selection Process

Applications will be evaluated for scientific and technical merit by appropriate Scientific Review Groups, in accordance with [NIH peer review policy and procedures \(https://grants.nih.gov/grants/guide/redirect.php?id=11154\)](https://grants.nih.gov/grants/guide/redirect.php?id=11154), using the stated [review criteria \(file:///C:/Users/mckenziene/AppData/Local/Microsoft/Windows/NetCache/Content.Outlook/13V4QPZR/Research%20Draft.doc#_1_.Criteria\)](https://grants.nih.gov/grants/guide/redirect.php?id=82418). Assignment to a Scientific Review Group will be shown in the eRA Commons.

As part of the scientific peer review, all applications will receive a written critique.

Applications may undergo a selection process in which only those applications deemed to have the highest scientific and technical merit (generally the top half of applications under review) will be discussed and assigned an overall impact score.

Applications will be assigned on the basis of established PHS referral guidelines to the appropriate NIH Institute or Center. Applications will compete for available funds with all other recommended applications submitted in response to this NOFO. Following initial peer review, recommended applications will receive a second level of review by the appropriate national Advisory Council or Board. The following will be considered in making funding decisions, consistent with applicable law.

- Scientific and technical merit of the proposed project as determined by scientific peer review.
- Availability of funds.
- Relevance of the proposed project to program priorities.

Please note that reviewers will not consider race, ethnicity, age, or sex of a researcher, award participant, or trainee, even in part, in providing critiques, scores, or funding recommendations. NIH will not consider such factors in making its funding decisions.

If the application is under consideration for funding, NIH will request "just-in-time" information from the applicant as described in the [NIH Grants Policy Statement Section 2.5.1. Just-in-Time Procedures \(https://grants.nih.gov/grants/guide/redirect.php?id=82418\)](https://grants.nih.gov/grants/guide/redirect.php?id=82418). This request is not a Notice of Award nor should it be construed to be an indicator of possible funding.

Prior to making an award, NIH reviews an applicant's federal award history in SAM.gov to ensure sound business practices. An applicant can review and comment on any information in the Responsibility/Qualification records available in SAM.gov. NIH will consider any comments by the applicant in the Responsibility/Qualification records in SAM.gov to ascertain the applicant's integrity, business ethics, and performance record of managing Federal awards per 2 CFR Part 200.206 "Federal awarding agency review of risk posed by applicants." This provision will apply to all NIH grants and cooperative agreements except fellowships.

3. Anticipated Announcement and Award Dates

After the peer review of the application is completed, the PD/PI will be able to access his or her Summary Statement (written critique) via the [eRA Commons \(https://grants.nih.gov/grants/guide/redirect.php?id=11123\)](https://grants.nih.gov/grants/guide/redirect.php?id=11123). Refer to Part 1 for dates for peer review, advisory council review, and earliest start date.

Information regarding the disposition of applications is available in the [NIH Grants Policy Statement Section 2.4.4 Disposition of Applications \(https://grants.nih.gov/grants/guide/redirect.php?id=82416\)](https://grants.nih.gov/grants/guide/redirect.php?id=82416).

Section VI. Award Administration Information**1. Award Notices**

A Notice of Award (NoA) is the official authorizing document notifying the applicant that an award has been made and that funds may be requested from the designated HHS payment system or office. The NoA is signed by the Grants Management Officer and emailed to the recipient's business official.

In accepting the award, the recipient agrees that any activities under the award are subject to all provisions currently in effect or implemented during the period of the award, other Department regulations and policies in effect at the time of the award, and applicable statutory provisions.

Recipients must comply with any funding restrictions described in [Section IV.6. Funding Restrictions](#). Any pre-award costs incurred before receipt of the NoA are at the applicant's own risk. For more information on the Notice of Award, please refer to the [NIH Grants Policy Statement Section 5. The Notice of Award \(https://grants.nih.gov/grants/policy/nihgps/HTML5/section_5/5_the_notice_of_award.htm\)](https://grants.nih.gov/grants/policy/nihgps/HTML5/section_5/5_the_notice_of_award.htm) and NIH Grants & Funding website, see [Award Process \(https://grants.nih.gov/grants/pre-award-process.htm#award\)](https://grants.nih.gov/grants/pre-award-process.htm#award).

Individual awards are based on the application submitted to, and as approved by, the NIH and are subject to the IC-specific terms and conditions identified in the NoA.

ClinicalTrials.gov: If an award provides for one or more clinical trials. By law (Title VIII, Section 801 of Public Law 110-85), the "responsible party" must register and submit results information for certain "applicable clinical trials" on the ClinicalTrials.gov Protocol Registration and Results System Information Website (<https://register.clinicaltrials.gov>)

(<https://register.clinicaltrials.gov/>)). NIH expects registration and results reporting of all trials whether required under the law or not. For more information, see <https://grants.nih.gov/policy/clinical-trials/reporting/index.htm> (<https://grants.nih.gov/policy/clinical-trials/reporting/index.htm>).

Institutional Review Board or Independent Ethics Committee Approval: Recipient institutions must ensure that all protocols are reviewed by their IRB or IEC. To help ensure the safety of participants enrolled in NIH-funded studies, the recipient must provide NIH copies of documents related to all major changes in the status of ongoing protocols.

Data and Safety Monitoring Requirements: The NIH policy for data and safety monitoring requires oversight and monitoring of all NIH-conducted or -supported human biomedical and behavioral intervention studies (clinical trials) to ensure the safety of participants and the validity and integrity of the data. Further information concerning these requirements is found at http://grants.nih.gov/grants/policy/hs/data_safety.htm and in the application instructions (SF424 (R&R) and PHS 398).

Investigational New Drug or Investigational Device Exemption Requirements: Consistent with federal regulations, clinical research projects involving the use of investigational therapeutics, vaccines, or other medical interventions (including licensed products and devices for a purpose other than that for which they were licensed) in humans under a research protocol must be performed under a Food and Drug Administration (FDA) investigational new drug (IND) or investigational device exemption (IDE).

2. Administrative and National Policy Requirements

The following Federal wide and HHS-specific policy requirements apply to awards funded through NIH:

- The rules listed at [2 CFR Part 200](https://www.ecfr.gov/current/title-2/subtitle-A/chapter-II/part-200) (<https://www.ecfr.gov/current/title-2/subtitle-A/chapter-II/part-200>), Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards.
- All NIH grant and cooperative agreement awards include the [NIH Grants Policy Statement](https://grants.nih.gov/grants/guide/uri_redirect.php?id=11120) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=11120) as part of the terms and conditions in the Notice of Award (NoA). The NoA includes the requirements of this NOFO. For these terms of award, see the [NIH Grants Policy Statement Part II: Terms and Conditions of NIH Grant Awards, Subpart A: General](https://grants.nih.gov/grants/guide/uri_redirect.php?id=11120) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=11120) and [Part II: Terms and Conditions of NIH Grant Awards, Subpart B: Terms and Conditions for Specific Types of Grants, Recipients, and Activities](https://grants.nih.gov/grants/guide/uri_redirect.php?id=11159) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=11159).
- If a recipient receives an award, the recipient must follow all applicable nondiscrimination laws. The recipient agrees to this when registering in SAM.gov. The recipient must also submit an Assurance of Compliance ([HHS-690](https://www.hhs.gov/sites/default/files/form-hhs690.pdf) (<https://www.hhs.gov/sites/default/files/form-hhs690.pdf>)). To learn more, see the [Laws and Regulations Enforced by the HHS Office for Civil Rights website](https://www.hhs.gov/civil-rights/for-providers/laws-regulations-guidance/laws/index.html) (<https://www.hhs.gov/civil-rights/for-providers/laws-regulations-guidance/laws/index.html>).
 - HHS recognizes that NIH research projects are often limited in scope for many reasons that are nondiscriminatory, such as the principal investigator's scientific interest, funding limitations, recruitment requirements, and other considerations. Thus, criteria in research protocols that target or exclude certain populations are warranted where nondiscrimination justifications establish that such criteria are appropriate with respect to the health or safety of the subjects, the scientific study design, or the purpose of the research. For additional guidance regarding how the provisions apply to NIH grant programs, please contact the Scientific/Research Contact that is identified in Section VII under Agency Contacts of this NOFO.

All federal statutes and regulations relevant to federal financial assistance, including those highlighted in [NIH Grants Policy Statement Section 4 Public Policy Requirements, Objectives and Other Appropriation Mandates](https://grants.nih.gov/grants/policy/nihgps/html5/section_4/4_public_policy_requirements_objectives_and_other_appropriation_mandates.htm), (https://grants.nih.gov/grants/policy/nihgps/html5/section_4/4_public_policy_requirements_objectives_and_other_appropriation_mandates.htm).

Recipients are responsible for ensuring that their activities comply with all applicable federal regulations. NIH may terminate awards under certain circumstances. See [2 CFR Part 200.340 Termination](https://www.ecfr.gov/current/title-2/subtitle-A/chapter-II/part-200/subpart-D/subject-group-ECFR86b76dde0e1e9dc/section-200.340) (<https://www.ecfr.gov/current/title-2/subtitle-A/chapter-II/part-200/subpart-D/subject-group-ECFR86b76dde0e1e9dc/section-200.340>) and [NIH Grants Policy Statement Section 8.5.2 Remedies for Noncompliance or Enforcement Actions: Suspension, Termination, and Withholding of Support](https://grants.nih.gov/grants/policy/nihgps/html5/section_8/8.5.2_remedies_for_noncompliance_or_enforcement_actions-suspension_termination_and_withholding_of_support.htm) (https://grants.nih.gov/grants/policy/nihgps/html5/section_8/8.5.2_remedies_for_noncompliance_or_enforcement_actions-suspension_termination_and_withholding_of_support.htm).

Successful recipients under this NOFO agree that:

Where the award funding involves implementing, acquiring, or upgrading health IT for activities by any funded entity, recipients and subrecipient(s) are required to: Use health IT that meets standards and implementation specifications adopted in 45 CFR part 170, Subpart B, if such standards and implementation specifications can support the activity. Visit <https://www.ecfr.gov/current/title-45/subtitle-A/subchapter-D/part-170/subpart-B> (<https://www.ecfr.gov/current/title-45/subtitle-A/subchapter-D/part-170/subpart-B>) to learn more.

Where the award funding involves implementing, acquiring, or upgrading health IT for activities by eligible clinicians in ambulatory settings, or hospitals, eligible under Sections 4101, 4102, and 4201 of the HITECH Act, use health IT certified under the ONC Health IT Certification Program if certified technology can support the activity. Visit <https://www.healthit.gov/topic/certification-ehrs/certification-health-it> (<https://www.healthit.gov/topic/certification-ehrs/certification-health-it>) to learn more.

Pursuant to the Cybersecurity Act of 2015, Div. N, § 405, Pub. Law 114-113, 6 USC § 1533(d), the HHS Secretary has established a common set of voluntary, consensus-based, and industry-led guidelines, best practices, methodologies, procedures, and processes.

Successful recipients under this NOFO agree that:

When recipients, subrecipients, or third-party entities have:

- 1) ongoing and consistent access to HHS owned or operated information or operational technology systems; and
- 2) receive, maintain, transmit, store, access, exchange, process, or utilize personal identifiable information (PII) or personal health information (PHI) obtained from the awarding HHS agency for the purposes of executing the award.

Recipients shall develop plans and procedures, modeled after the [NIST Cybersecurity framework](https://www.nist.gov/cyberframework) (<https://www.nist.gov/cyberframework>), to protect HHS systems and data. Please refer to [NIH Post-Award Monitoring and Reporting](https://grants.nih.gov/grants/post-award-monitoring-and-reporting.htm) (<https://grants.nih.gov/grants/post-award-monitoring-and-reporting.htm>) for additional information.

Cooperative Agreement Terms and Conditions of Award

Not Applicable

3. Data Management and Sharing

Consistent with the 2023 NIH Policy for Data Management and Sharing, when data management and sharing is applicable to the award, recipients will be required to adhere to the Data Management and Sharing requirements as outlined in the [NIH Grants Policy Statement](https://grants.nih.gov/grants/policy/nihgps/html5/section_8/8.2.3_sharing_research_resources.htm#Data) (https://grants.nih.gov/grants/policy/nihgps/html5/section_8/8.2.3_sharing_research_resources.htm#Data). Upon the approval of a Data Management and Sharing Plan, it is required for recipients to implement the plan as described.

4. Reporting

When multiple years are involved, recipients will be required to submit the [Research Performance Progress Report \(RPPR\)](https://grants.nih.gov/grants/rppr/index.htm) (<https://grants.nih.gov/grants/rppr/index.htm>) annually and financial statements as required in the [NIH Grants Policy Statement Section 8.4.1 Reporting](https://grants.nih.gov/grants/policy/nihgps/html5/section_8/8.4.1_reporting.htm) (https://grants.nih.gov/grants/policy/nihgps/html5/section_8/8.4.1_reporting.htm). To learn more about post-award monitoring and reporting, see the NIH Grants & Funding website, see [Post-Award Monitoring and Reporting](https://grants.nih.gov/grants/guide/uri_redirect.php?id=82428) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=82428).

A final RPPR, invention statement, and the expenditure data portion of the Federal Financial Report are required for closeout of an award, as described in the [NIH Grants Policy Statement Section 8.6 Closeout](https://grants.nih.gov/grants/guide/uri_redirect.php?id=82420) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=82420). NIH NOFOs outline intended research goals and objectives. Post award, NIH will review

and measure performance based on the details and outcomes that are shared within the RPPR, as described at 2 CFR Part 200.301.

Section VII. Agency Contacts

We encourage inquiries concerning this funding opportunity and welcome the opportunity to answer questions from potential applicants.

Application Submission Contacts

eRA Service Desk (Questions regarding ASSIST, eRA Commons, application errors and warnings, documenting system problems that threaten submission by the due date, and post-submission issues)

Finding Help Online: <https://www.era.nih.gov/need-help> (<https://www.era.nih.gov/need-help>) (preferred method of contact)

Telephone: 301-402-7469 or 866-504-9552 (Toll Free)

General Grants Information (Questions regarding application instructions, application processes, and NIH grant resources)

Email: GrantsInfo@nih.gov (<mailto:GrantsInfo@nih.gov>) (preferred method of contact)

Telephone: 301-480-7075

Grants.gov Customer Support (Questions regarding Grants.gov registration and Workspace)

Contact Center Telephone: 800-518-4726

Email: support@grants.gov (<mailto:support@grants.gov>)

Scientific/Research Contact(s)

Richard A. Jenkins, Ph.D.

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Email: jenkinsri@mail.nih.gov (<mailto:jenkinsri@mail.nih.gov>)

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National Institute on Drug Abuse (NIDA)

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Email: mandlerr@mail.nih.gov (<mailto:mandlerr@mail.nih.gov>)

Peer Review Contact(s)

Examine your eRA Commons account for review assignment and contact information (information appears two weeks after the submission due date).

Financial/Grants Management Contact(s)

Chief Grants Management Officer

National Institute of Drug Abuse (NIDA)

Email: nidagmbemail@nida.nih.gov (<mailto:nidagmbemail@nida.nih.gov>)

Section VIII. Other Information

Recently issued trans-NIH [policy notices](https://grants.nih.gov/grants/guide/uri_redirect.php?id=11163) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=11163) may affect your application submission. A full list of policy notices published by NIH is provided in the [NIH Guide for Grants and Contracts](https://grants.nih.gov/grants/guide/uri_redirect.php?id=11164) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=11164). All awards are subject to the terms and conditions, cost principles, and other considerations described in the [NIH Grants Policy Statement](https://grants.nih.gov/grants/guide/uri_redirect.php?id=11120) (https://grants.nih.gov/grants/guide/uri_redirect.php?id=11120).

Authority and Regulations

Awards are made under the authorization of Sections 301 and 405 of the Public Health Service Act as amended (42 USC 241 and 284) and under Federal Regulations 42 CFR Part 52 and 2 CFR Part 200.

[Weekly TOC for this Announcement](https://grants.nih.gov/grants/guide/WeeklyIndex.cfm?01-10-25) (<https://grants.nih.gov/grants/guide/WeeklyIndex.cfm?01-10-25>)

[NIH Funding Opportunities and Notices](https://grants.nih.gov/grants/guide/index.html) (<https://grants.nih.gov/grants/guide/index.html>)



(<https://www.hhs.gov/>) Department of Health and Human Services (HHS)



(<https://www.usa.gov/>)

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